

information ENRICHMENT series

New Business and Underwriting

Module 3 – Underwriting and Risk Assessment (Medical)

MODULE OVERVIEW

GETTING TO KNOW YOUR NB & UW PARTNERS

01

*Table of Organization
Sections & Assignments
NB & UW Personnel Demographics
Authority limits*

UNDERSTANDING THE UNDERWRITING PROCESS

02

*Insurance Risk Selection and the
Premium
Equal Treatment for Equal Risks
Purpose of Underwriting
The Underwriting Process
Factors that can Prolong the Process*

TRANSACTIONS TO BE UNDERWRITTEN

03

*New Business
Reinstatement
Policy Change Request (addition of
benefits, request review or rating,
change to FPU, increase in coverage)
Review of claims during
contestability period*



UNDERWRITING COMMON IMPAIRMENTS

04

*Height and Weight beyond the
"normal range"
Hypertension
Adverse Family History
Diabetes
Elevated Lipids/Metabolic Syndrome*

WORKSHOP

05

Sample Case Studies

SCORECARD

06



15 Warm Bodies

Tina Dizon, PCS, FLMI, FALU
Chief NB & UW Officer
Date of Entry: April 1995



Lara Miguel, ACS, FLMI, FALU
Manager 3
SLOCPI Territory 1
Date of Entry: Feb 1997



Tere Gravador, PCS, AALU
Manager 1
Team 1
Date of Entry: Oct 2011



Anne Baello
FALU, 4B
Date of Entry: Nov 2012



Dianne Carmela
San Agustin, 5A
Date of Entry: June 2014



Kathleen Gae
Arandia, 4A
Date of Entry: June 2014



Marc Conrad
Sison, ACS, 4A
Date of Entry: Apr 2017



Sha Tejero, ACS, ALMI, FALU
Manager 1
Team 2
Date of Entry: Jun 2007



Ayn Ramirez
ACS, ALMI,
AALU, 4B
Date of Entry: July 2011



Jan Camille
Gondales, 4B
Date of Entry: Jan 2015



Mark Angelo
Suarez, 4A
Date of Entry: Apr 2016



Precious
Bandoquillo, 4A
Date of Entry: Apr 2017



Aby Elijah
Bernardino,
4A (T)
Date of Entry: Aug 2018



Minette Marquez,
ACS, AALU
Supervisor
Team 3
Date of Entry: Jun 2013



John Vincent
Escalante, 4B
Date Entry in NB &
UW: Feb 2016



Erwin U. Imperio
4B (T)
Date Entry in NB &
UW: June 2018



SLOCPI  Territory 1 - Underwriters



02 UNDERSTANDING THE UNDERWRITING PROCESS

Definition of UNDERWRITING

Characteristics of Fair UNDERWRITING DECISION

Assessing anticipated
MORBIDITY AND MORTALITY **RISKS**

**Risk
Selection**

**Risk
Classification**

DECISION

Equitable to the client

Deliverable by the advisor

Profitable to the company



1. COLLECTION OF INFORMATION

Underwriter gathers information from various sources of client's data

2. ANALYZATION AND EVALUATION

The collected information are used to determine client's appropriate risk classification --- in terms of probability of disability or death

3. IDENTIFICATION OF OPTIONS

The underwriting decision will be matched based on client's risk class and appropriate premium rates



02

UNDERWRITING PROCESS



BASIC RULE of UNDERWRITING

UNDERWRITE the additional risk to be assumed by the company

SOURCES OF INFORMATION



Aside from the previously mentioned sources of information, underwriters also **check the following:**

- Residence Address
- Employment Address
- Beneficiary
- Previous insurance coverage



EVALUATION

1

Personal
Information

2

Medical
Information

3

Financial
Information

Ability to Pay
(Persistency)
Appropriate Need
for Insurance

ROUTINE REQUIREMENTS are determined based on the amount applied and total coverage within the past 24 months.

MEDICAL EXAM AND SPECIAL TESTS

BLOOD TEST COMPOSITION

SL12 OR
SUN LIFE PACKAGE 1

SL13 OR
SUN LIFE PACKAGE 2

SUN LIFE PACKAGE 3

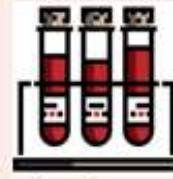
REFLEX TESTS



Urinalysis



Cotinine*



Blood Tests



ECG



Treadmill



X-ray

SPECIAL TESTS FEES AND REIMBURSEMENT

Special tests should be completed by **our accredited hospitals and laboratories.**

We will no longer reimburse costs of special tests done in non-accredited hospitals and laboratories.

DIPSTICK URINALYSIS AND MICROURINALYSIS

Dipstick urinalysis is done by paramedics or some examiners and a part of medical exam. **This is not the special test urinalysis.** If this is not done, *client will be subjected to micro urinalysis.*

Special test urinalysis is called as **micro urinalysis** and **always done in accredited laboratories.**

This is applicable only for insureds aged 16 and above.

SOURCES
OF INFORMATION



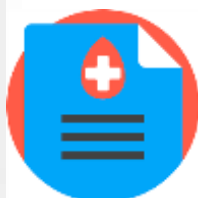
DISCRETIONARY REQUIREMENTS are requirements ordered depending on the results of the initial assessment of the application.

NEW BUSINESS REVIEW ASSOCIATES OR ANALYSTS

Amendment of Application



- Amendment of Application Form
- Advisor's Report



UNDERWRITERS

- Amendment of Application Form
- Advisor's Report
- Attending Physician Statement (APS)
- Copy of medical records
- Special tests – blood test, urinalysis, ECG, Treadmill or X-ray

APS Guidelines:

1. If complete name and address of doctor has been provided and hospital or clinic is within Metro Manila, we can ask our Messenger to obtain APS.
2. In case the doctor **will charge more than PHP 1,000, inform your Underwriter if you should proceed to obtain the report**, or if the Underwriter can just ask for an alternative requirement.

REMINDERS

Underwriters reserve the right to require additional evidences depending on the evaluation of the client's medical condition.

Selling outside the Philippines is not allowed.

Applications signed before the coding date cannot be processed and will not be accepted.

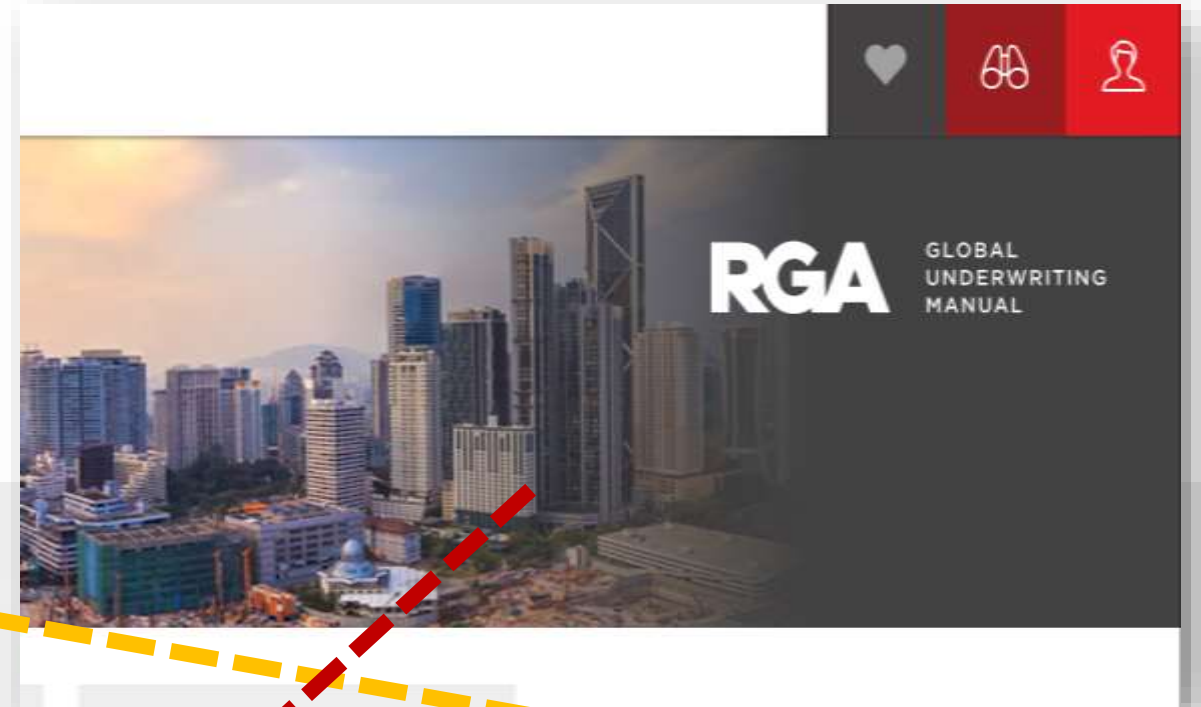
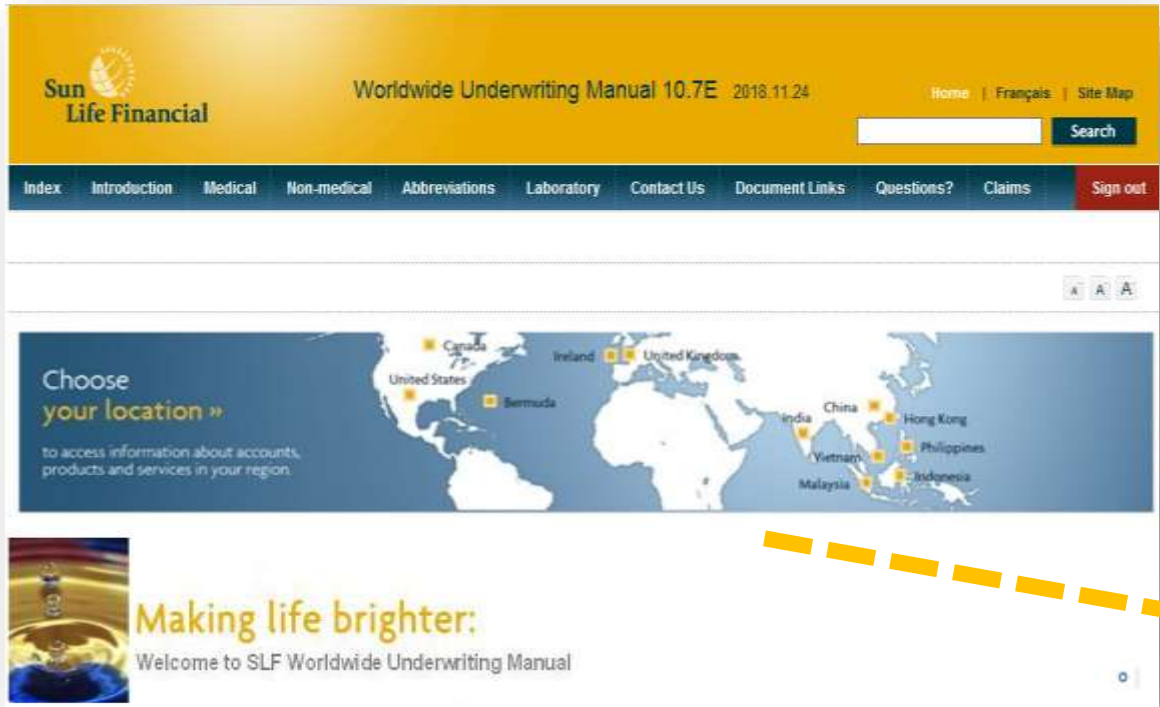
Non-medical authority limits cannot be delegated.

If there are sharing Advisors, the non-medical authority limit of **the one who completed the application and conducted the non- medical interview will be followed.**

We can allow Advisors to sign or witness the application of their immediate relatives.

SOURCES
OF INFORMATION



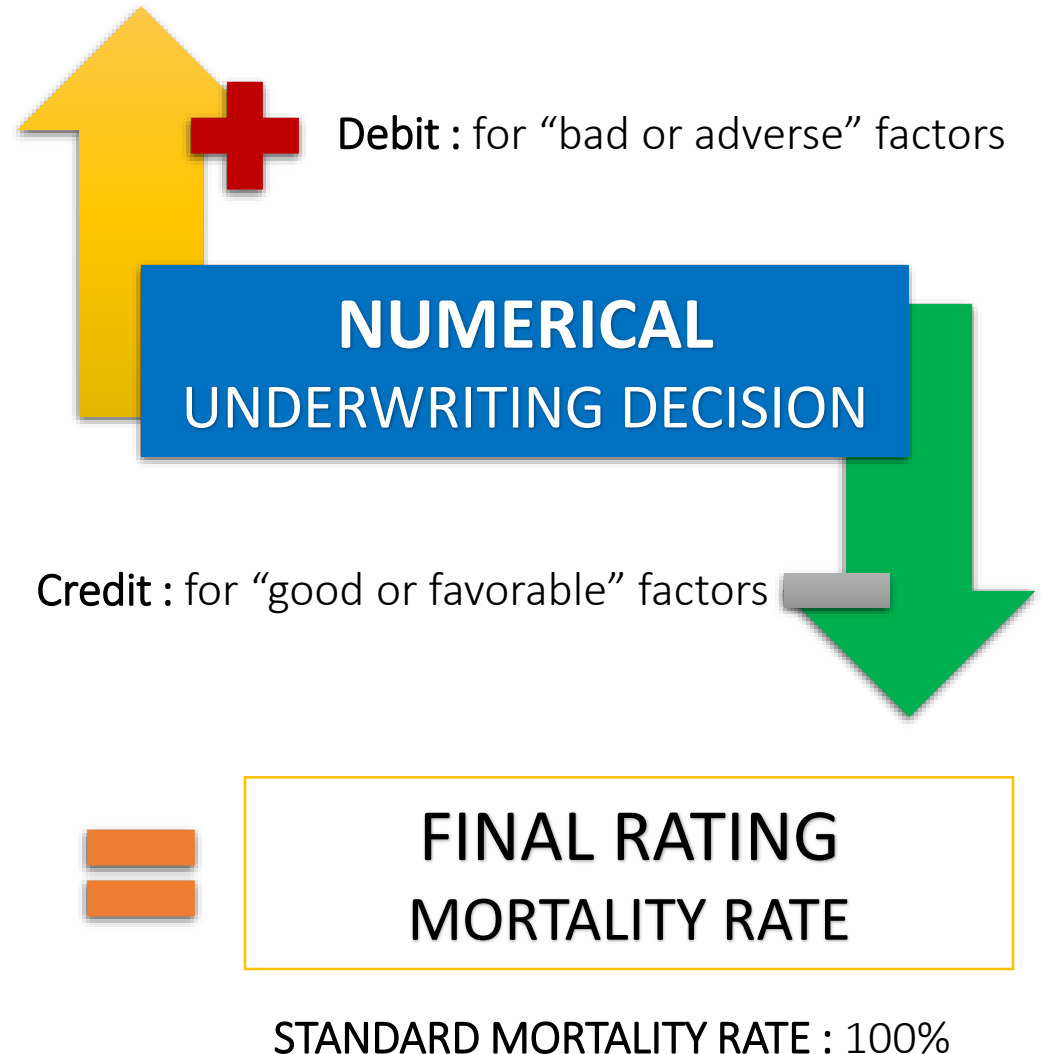


UNDERWRITING
MANUALS



Basis of Underwriting Decision

- Sources of information
- Underwriting Manuals
- Inputs of Medical Officers
- Discussions with or referrals to:
 - Senior Underwriters
 - Underwriting Managers
 - Underwriting Consultant
 - Chief Underwriter
 - Corporate Underwriters
 - Reinsurers



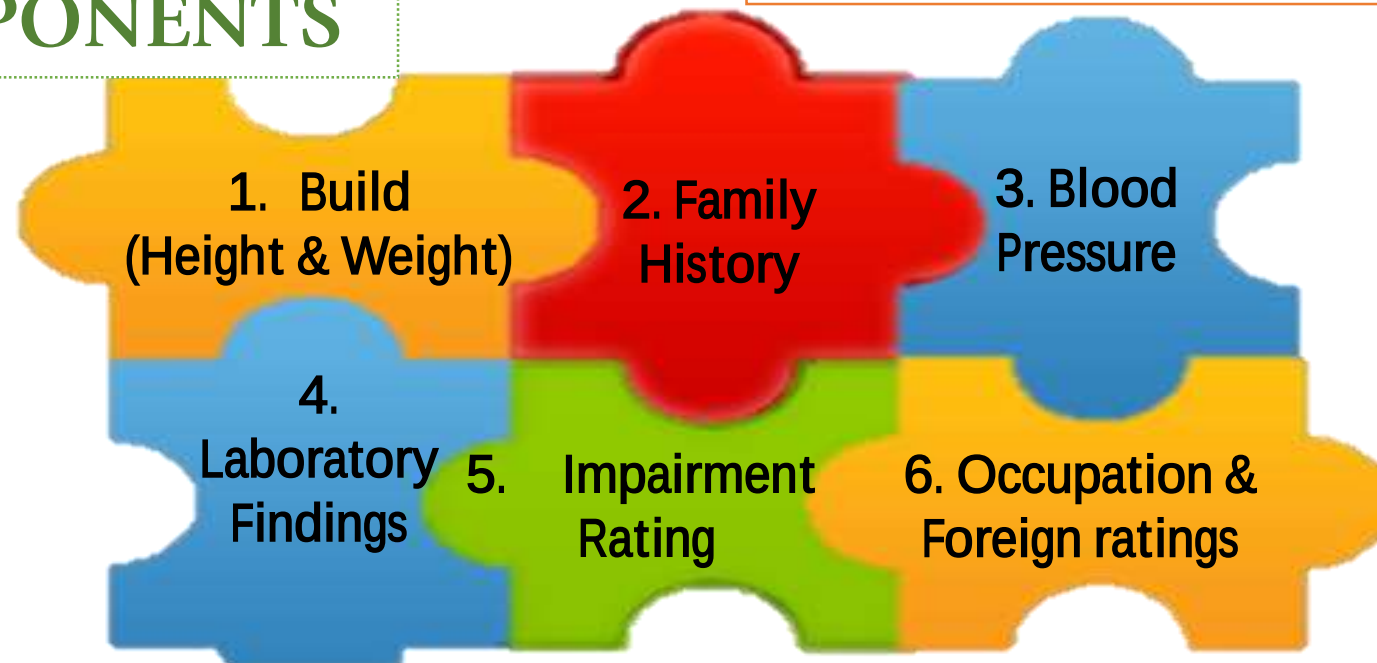
Basis of Underwriting Decision

mortality rate

– end result of underwriting assessment

****Standard Mortality:**
100%

COMPONENTS

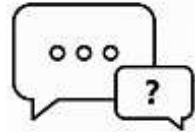


Factors that Prolong the Underwriting Process

Completion and consistency of answers on application form and other documents



Validation of vague or general declaration on client's medical history



Speed of submission, request to extend deadline or waive requirements



Client is already out of the country for travel, to study or work -- on-board.



UNDERWRITING
PROCESS



Risk Class and Types of Decision

STANDARD

anticipated mortality risk is regarded as average and with average premium rate

SUBSTANDARD

anticipated mortality risk is higher than the norm and with higher premium rate

POSTPONE

medical findings are not yet properly assessed or diagnosed

DECLINE

anticipated extra mortality risk is so significant and we cannot make an offer

NPW

applications are not proceeded with

NTU

cancellation of application after underwriting decision has been rendered

Cases not approved by NBREV Associates and Analysts will be queued for **INITIAL UNDERWRITING APPRAISAL**.
Queuing is based on amount applied for Sun Life and total with other companies



Underwriter will perform initial appraisal

If case is within underwriter's approval authority limit, requirements may be asked or decision may be issued already from this end.



Sr. Underwriter will perform subsequent appraisal

If case is beyond underwriter's approval authority limit, the application will be referred to a higher underwriter for second opinion.

Additional requirements might be asked or decision may be issued from this end.



Referral to Chief Underwriter

Complex, jumbo and high risk cases are referred to Chief UW for second opinion.



Discussion with Medical Officer

Underwriters may also seek the opinion of Medical Officers for cases, validation of medical records and findings.



Referral to UW Consultant, Corp. UW or Reinsurer

Unusual medical history, complicated, high net worth and jumbo cases may be referred to our Consultant, Corp. UW, or Reinsurer for inputs and comments

UNDERWRITING

REFERRAL PROCESS



TEST Your Knowledge!

Guess the appropriate underwriting decision.



**SHORT
SECTION REVIEW**

Benjie applied for Sun Fit and Well Advantage 10 with Ps 2M face amount, TDB and ADB. He has no medical history and adverse family history. File was referred to underwriter in view of occupation. Per checking, he is a military pilot.

Close as DECLINED case

Ana is a work-at-home new mom. She used to smoke back in college but has stopped as soon as she learned she was pregnant, and has not smoked since then. She realized that she wants to get protection as well as start saving for her son's education when she decided to quit her regular work.

Approve at STANDARD RATE

Marlo bought a Maxiprime policy which was approved after 2 days. Within 1 week from receiving the policy contract, he learned that his mom was confined in the hospital due to some serious illness and needs his financial support to address the medication costs and expenses. With this in mind, he decided to cancel his application.

Application is considered as NTU

TEST Your Knowledge!

Guess the appropriate underwriting decision.



SHORT
SECTION REVIEW

Brian is a young professional who loves the outdoors. He inquires about Sun FlexiLink, and would like to have Ps. 2M coverage. He engages in scuba diving at times and marathon races. He tries to keep himself fit, and does not take on any vices, except for occasional drinks with friends. He does so because he doesn't want to end up like his uncle who lost his battle to colon cancer

Approve at SUBSTANDARD RATE

Financial Advisor Myra submitted Mrs. De Vera's application six weeks ago. One week after, Myra received some additional requirements from Underwriting about the application, which she relayed to the client. It has been a month now since Underwriting's request, and Mrs. De Vera hasn't submitted any yet.

Close as NPW

- New Business Applications
- Reinstatement
- Policy change requests after policy issue, such as:
 - Addition of benefits
 - Requests for review of rating
 - Change to fully-paid up life
 - Increase of insurance coverage
- Review of claims file during contestability period

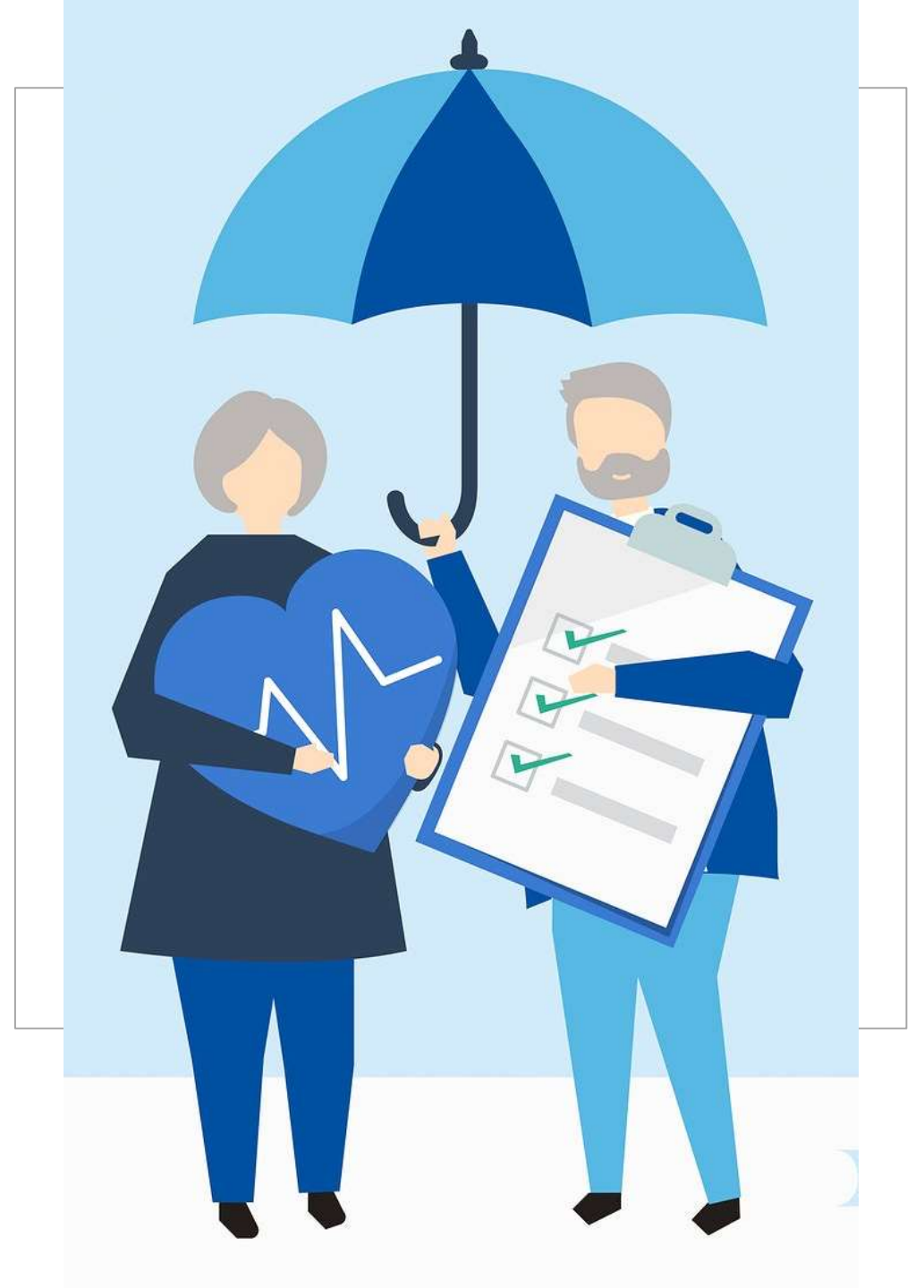


03

TRANSACTIONS TO BE
UNDERWRITTEN

BASIC RULE of UNDEWRITING is to underwrite the additional risk to be assumed by the company.

- Abnormal height and weight
- Hypertension
- Adverse Family History
- Diabetes
- Elevated Lipids/Metabolic Syndrome



Height and Weight Risk Assessment Guide

We evaluate insured's height and weight in relation to each other.

Once build is obtained, we compare it on build table which indicates average weights for various heights.

This table is based on established studies of height and weight.

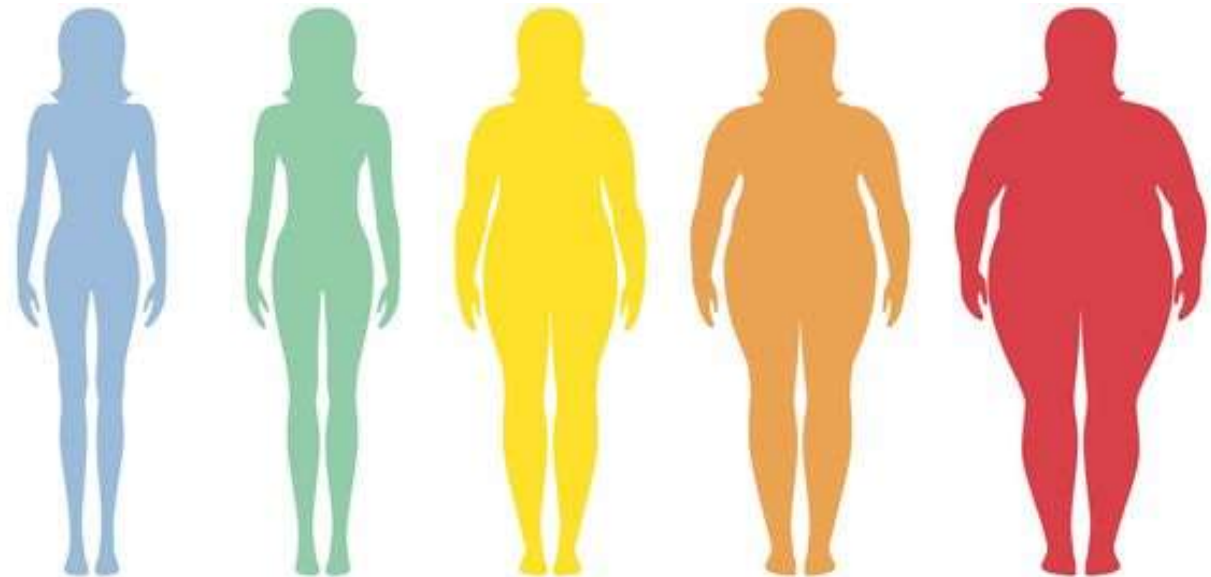
Body Mass Index or BMI is used to measure the body fat based on person's height and weight.

Illnesses associated with overweight

Heart diseases
Diabetes
Hypertension
Dyslipidemia
Sleep Apnea
Arthritis
Menstrual Irregularities

Illnesses associated with underweight

Psychiatric problems
Eating disorders
Digestive disorders
Cancers
Alcoholism
Respiratory disorders



Client profile:

35 years old

Male

Non - Smoker

Call Center Agent

Earning Ps. 780k/annum

Basic Information:

- No adverse family history
- Most recent APE results are all within normal limits
- History of appendectomy 20 years ago
- 5'10", 280 lbs.

Thoughts to ponder:

Among the given client information, which is an adverse finding?
What will be the underwriting approach?

CASE NUMBER 1



Sample Build Table

Height	Underweight Debit		Ave. Wt.	Overweight Debit			
FT IN	50	25	0	25	50	75	100
5'8"	99 – 104	105 – 111	112 – 217	218 – 237	238 – 256	257 - 270	271 – 283
5'9"	102 – 108	109 – 114	115 – 223	224 – 244	245 – 264	265 – 278	279 – 291
5'10"	105 – 111	112 – 118	119 – 230	231 – 251	252 – 272	273 – 286	287 – 300

Answer:

There is a debit of 75 or +75.

This will be added to base mortality rating of 100%.

Underwriting decision will be substandard, with a rating of 175%, to cover risk related to above average weight.

Client profile:

27 years old

Female

Non - Smoker

Flight Attendant

Earning Ps 1.2M/annum

Basic Information:

- No adverse family history
- Most recent APE results are all within normal limits
- No adverse personal medical history
- 5'10", 124 lbs.

Thoughts to ponder:

Among the given client information, is there an adverse finding?
What will be the underwriting approach?

CASE NUMBER 2



Sample Build Table

Height	Underweight Debit		Ave. Wt.	Overweight Debit			
FT IN	50	25	0	25	50	75	100
5'8"	99 – 104	105 – 111	112 – 217	218 – 237	238 – 256	257 – 270	271 – 283
5'9"	102 – 108	109 – 114	115 – 223	224 – 244	245 – 264	265 – 278	279 – 291
5'10"	105 – 111	112 – 118	119 – 230	231 – 251	252 – 272	273 – 286	287 – 300

Answer:

Insured's build falls under the average range

There's no additional debit to base mortality rating of 100%.

Underwriting decision will be standard.

Client profile:

18 years old

Female

Non - Smoker

Student

Financially dependent to parent

Basic Information:

- No adverse family history
- With recent hospitalization due to abdominal pain
- 5'10", 105 lbs.

Thoughts to ponder:

Among the given client information, which is/are an adverse finding(s)?

What will be the underwriting approach?

Sample Build Table

Height	Underweight Debit		Ave. Wt.	Overweight Debit			
FT IN	50	25	0	25	50	75	100
5'8"	99 – 104	105 – 111	112 – 217	218 – 237	238 – 256	257 – 270	271 – 283
5'9"	102 – 108	109 – 114	115 – 223	224 – 244	245 – 264	265 – 278	279 – 291
5'10"	105 – 111	112 – 118	119 – 230	231 – 251	252 – 272	273 – 286	287 – 300

Answer:

There is a debit of 50 or +50.

This will be added to base mortality rating of 100%.

Underwriting decision will be substandard, with a rating of 150% ,to cover risk related to low weight.



Blood Pressure Risk Assessment Guide

Measurement of blood pressure is a useful tool in evaluation of an individual's health.



Systolic Blood Pressure is the pressure when the heart ventricles contract.

Diastolic Blood Pressure is the pressure when the heart ventricles are relaxed.

- Normal blood pressure (BP): systolic <120 mmHg and diastolic <80 mmHg
- Prehypertension: systolic 120 to 139 mmHg or diastolic 80 to 89 mmHg
- Hypertension: systolic >140 mmHg and/or diastolic >90 mmHg

Client profile:

33 years old

Male, Smoker

Data Scientist

Earning Ps. 1.0M/annum

Plan and coverage details:

Applying for:

- Sun Smarter Life Classic
Regular Pay – Ps. 1M
- TDB/ADB

Basic Information:

- No adverse family history
- APE done 3 weeks ago,
results are all within normal
limits except for elevated BP
reading of 150/100 mmHg.
- Advised by Company
Physician to take Losartan
50mg/tab daily for 30 days
- 5'8", 150 lbs.

CASE NUMBER 4

Medical exam was requested

- Exam done 3 days ago.
- BP reading on 3 counts is 160/100 mmHg.

Sample BP Table

SYSTOLIC				
DIASTOLIC	AGE	TO 140	141 - 150	151 -160
91 - 95	15 – 39	0	10	35
	40 – 49	0	0	15
	50 – 59	0	0	0
	60 >	0	0	0
96 - 100	15 – 39	25	35	60
	40 – 49	5	15	30
	50 – 59	0	5	15
	60 >	0	0	5

Underwriting decision is
substandard to cover risk
of elevated blood
pressure reading.

Answer:

There is a debit of 60 or +60 .

This will be added to base mortality rating of 100%.

Client profile:

36 years old

Male, Non - smoker

Company Driver

Earning Ps. 360k/annum

Plan and coverage details:

Applying for:

- Sun Flexilink – Ps. 500k
- TDB, ADB, CIB – Ps. 500k
- HIB – Ps. 700/day

Basic Information:

- No adverse family history
- APE done 3 months ago, results are all within normal limits except for elevated BP reading of 150/100 mmHg.
- Advised by Company Physician to take Amlodipine 20mg/tab daily for 30 days
- 5'8", 135 lbs.

Medical exam was requested

- Exam done 1 week ago.
- BP reading on 3 counts is 120/80 mmHg.

Sample BP Table

				SYSTOLIC
DIASTOLIC	AGE	TO 140	141 - 150	151 -160
To 80	15 – 39	0	0	0
	40 – 49	0	0	0
	50 – 59	0	0	0
	60 >	0	0	0

Answer:

There is no debit for insured's BP reading.

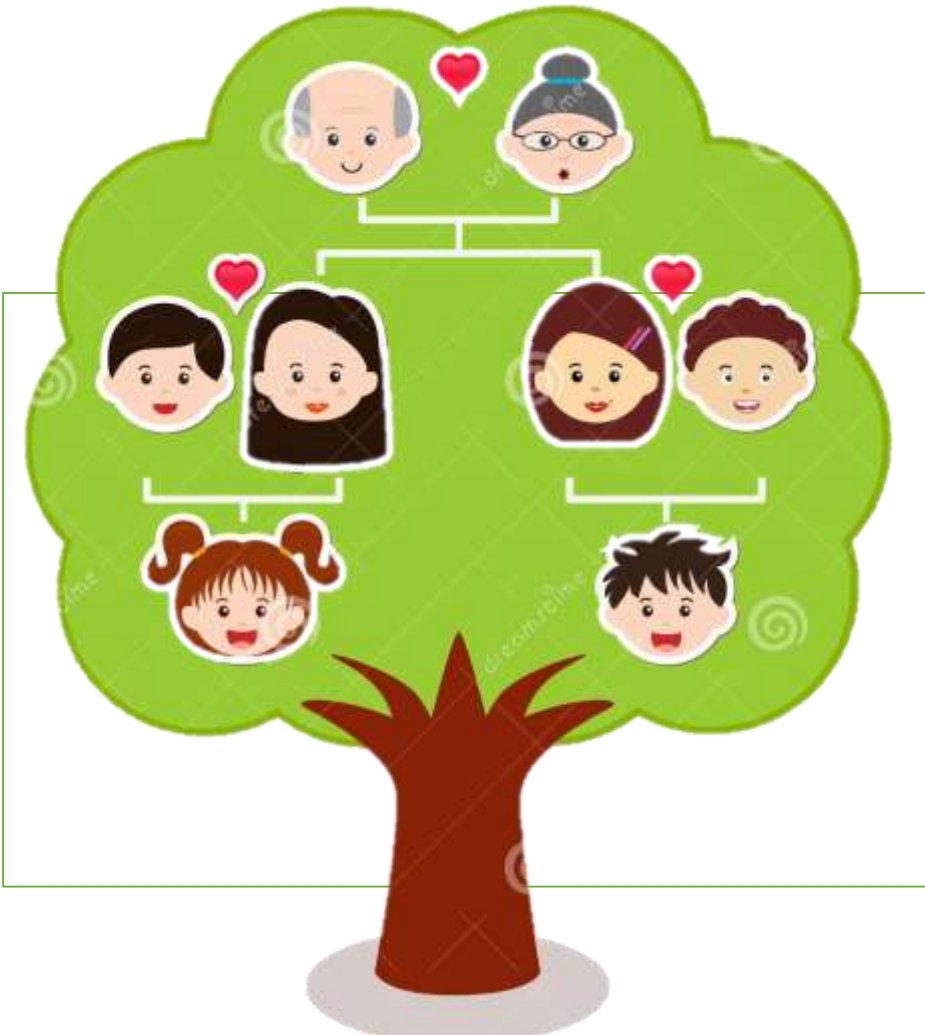
Mortality rating is maintained at 100%.

Underwriting decision is standard.



Family History Risk Assessment Guide

Family history is considered as a factor to increase the risk of developing certain disorder.



Examples of highly familial disease:

- Heart disease (Cardiomyopathy)
- Cancer
- Polycystic Kidney Disease
- Diabetes

Common underwriting case:

Family history of diabetes

Client profile:

29 years old

Female

Non - Smoker

Store Manager

Earning Ps. 800k/Annum

Plan and coverage details:

Applying for:

- Sun Maxlink Prime – Ps. 1M
- TDB, ADB, CIB – Ps. 500k
- HIB – Ps. 2K/day

Basic Information:

- Recent APE within normal limits, history of dengue fever, 10 years ago.
- 5'2, 120 lbs.
- Family history questions answered as “yes” and with details provided.

CASE NUMBER 6



Reason for referral to underwriter: Family history

Father – diagnosed with DM at 55

Brother – diagnosed with DM at 31

Sample Family History Declaration:

94. Family History: If space is insufficient, use Amendment of Application.

Life to be Insured					Applicant/Owner of WPD/WPDD				
Family Member	Age (If Alive)	Health Condition/ Medical Diagnosis	Age at Death	Cause of Death	Family Member	Age (If Alive)	Health Condition/ Medical Diagnosis	Age at Death	Cause of Death
Father	69	Diabetic			Father				
Mother	64	Kidney failure, MDS			Mother				
Brothers	31, 35, 37, 40	31 - diabetic			Brothers				
Sisters	34	IN GOOD HEALTH			Sisters				

Instructions: Underline conditions being referred to. Provide further details on the next page for any “Yes” answer.

95. Has any of your parents, brothers or sisters, whether living or dead, been diagnosed with breast or colon cancer or other types of cancer, heart disease, cardiomyopathy, stroke, diabetes, muscular dystrophy, Alzheimer's disease, Parkinson's disease, polycystic kidney disease, or any other hereditary disorder before age 60? Indicate age at onset of illness.

☒ Yes ☐ No

FATHER @ 55 : diabetic

SL UW Manual

Diabetes

Note: The guidelines below only apply for death or disease in parent or sibling prior to age 60

Life

TDB

CI

Type 2

1 family member

0

0

2 or more family members*

0

25*

RGA GUM Manual

Family History of Diabetes Mellitus

Personal Note

Description Life Accidental Death Benefit Critical Illness Stages Products Disability Income Waiver of Premium Total and Permanent Disability Hospital & Surgical Cash

Hospital & Surgical Cash

RATING

Ignore any family history diagnosed before age 20 or after age 60

Age of applicant

≥60

0*

<60

Number of relatives diagnosed between ages 20-60 (excluding identical twin)

One 1st degree relative

0

Two 1st degree relatives

25

Answer:

CIB +25 and HIB +25 (+50)

Underwriting decision is
substandard and with offer
for CIB and HIB riders.

Sample Offer Form and Proposal

ACCEPTED

19 2019

Sun Life Financial

Acceptance of Offer

Please PRINT clearly.
Use BLACK ink.

In this form, you and your refer to the person being insured and the applicant or the planholder who is named in the application as the buyer of the pre-need plan, whichever is applicable, while we, us, our and the Company refer to Sun Life of Canada (Philippines), Inc. or Sun Life Financial Plans, Inc. Both are members of the Sun Life Financial group of companies. Sun Life of Canada (Philippines), Inc. is the Insurance Provider contracted by Sun Life Financial Plans, Inc. to underwrite the insurance benefits under the chosen pre-need program.

This is in connection with the application for life insurance under:

Serial No. of Application/Policy or Plan No.

1

General Information

Life to be insured/Planholder (Last Name, First Name, Middle Name)

Client No.

Agent/Sales Consultant

ISO/HBO

2

Amendments

The application for the above numbered policy/plan is hereby amended or corrected as indicated below. A copy of this Acceptance of Offer, the original of which is to be retained by the Company, shall apply to any policy/plan issued thereon.

RATE AGE: 29 y/o

PART I OF APPLICATION (SECTION 2.1) QUESTION NO.1 : BASIC PLAN

SUN MAXILINK PRIME (NON SMOKER) FOR PS.1,000,000.00

WITH TOTAL DISABILITY BENEFIT

WITH ACCIDENTAL DEATH BENEFIT FOR PS.1,000,000.00

WITH CRITICAL ILLNESS BENEFIT FOR PS.500,000.00

WITH HOSPITAL INCOME BENEFIT FOR PS.2,000.00 PER DAY

SPECIAL INSTRUCTIONS

CRITICAL ILLNESS BENEFIT AT 1.25 TIMES THE REGULAR RATE

HOSPITAL INCOME BENEFIT AT 1.5 TIMES THE REGULAR RATE

Variable Life Insurance Proposal

Sun Life Financial

In the Philippines, insurance products are being offered through Sun Life of Canada (Philippines), Inc., a member of the Sun Life Financial group of companies. In this proposal, you and your refer to the policy owner while we, us, our and the Company refer to Sun Life of Canada (Philippines), Inc.

Proposal Information

On the Life of : Jenin Rosanne Velasquez

Age Last Birthday : 29 (Female)

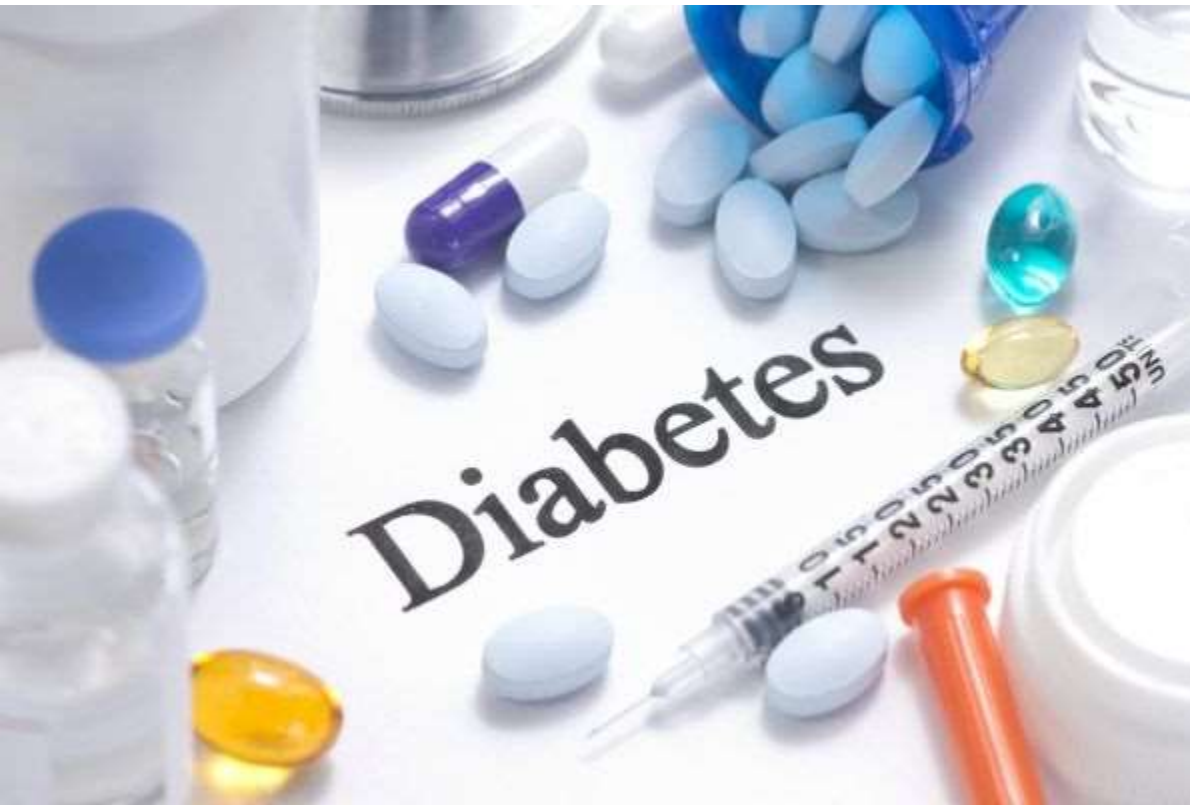
Currency : Philippine Peso

Basic Plan	Benefit Amount	Regular Premium
Sun MaxiLink Prime - Non-Smoker	Ps 1,000,000.00	Ps 46,920.00
Additional Benefits		
Total Disability Benefit (Waiver of Premium) - TDB		Ps 1,250.00
Accidental Death Benefit - ADB	Ps 1,000,000.00	Ps 1,340.00
Critical Illness Benefit - CIB	Ps 500,000.00	Ps 2,356.25
Hospital Income Benefit - HIB	Ps 2,000.00 /day	Ps 7,380.00
Class: With Extra Premium	Total Initial Annual Regular Premium	Ps 59,246.25
CIB - 1.25x up to year 41	Annual Excess Premium (to be Billed Regularly)	Ps 6,753.75
HIB - 1.5x up to year	Total Initial Annual Premium (to be Billed Regularly)	Ps 66,000.00
	Excess Premium (One-time)	Ps 0.00

Case settled in substandard decision.

Diabetes Risk Assessment Guide

Diabetes mellitus (DM) is a group of common, chronic metabolic disorders characterized by elevated blood glucose levels (hyperglycemia).



- Diabetes is associated with **serious health complications** affecting multiple organ systems including the heart, kidneys, eye, brain, and limbs.

Client profile:

49 years old

Male

Non - Smoker

Electrical Engineer

Earning Ps. 800k/Annum

Plan and coverage details:

Applying for:

- Sun Flexilink – Ps. 1M
- TDB, ADB - Ps. 1M

Basic Information:

- Diagnosed with diabetes 2 years ago
- No significant family history
- 5'2, 162 lbs.

Glimeperide and Metformin are Oral hypoglycemic drugs

98. Has any of your parents, brothers or sisters, whether living or dead, been diagnosed with breast, colon, ovarian, rectal, or other types of cancer, heart disease, cardiomyopathy, stroke, diabetes, muscular dystrophy, Alzheimer's disease, Parkinson's disease, polycystic kidney disease, or any other hereditary disorder before age 60? Indicate age at onset of illness.		☐ Yes ☒ No
99. Are you currently taking any medication? Name of medication <u>Glimeperide + Metformin</u> Doctor's name and clinic address <u>Pharidel, Bul.</u>		☒ Yes ☐ No
100. Height and Weight Information	Present height (indicate unit of measurement) Present weight (indicate unit of measurement)	<u>5'7 inches</u> <u>72 kgs</u>
101. Has there been a weight change of more than 10 pounds (4.5 kilos) within the last 12 months? If "Yes," provide details. Life to be Insured: Reason: _____ Gained _____ lbs Lost _____ lbs Applicant/Owner of WPD/WPDD: Reason: _____ Gained _____ lbs Lost _____ lbs		☐ Yes ☒ No
102. Many people during their lifetime will experience or be treated for medical conditions. Please let us know which of the following you have had, or been told you had, or sought advice or treatment for: a. high blood pressure, chest pain/discomfort, heart murmur, rheumatic fever, stroke, aneurysm, circulatory or heart disorder? b. diabetes, sugar in the urine, thyroid or other glandular (endocrine) disorder? c. kidney, bladder, or urinary disorder/infection, sexually transmitted disease, reproductive organ or prostate disorder?		☒ Yes ☐ No ☒ Yes ☐ No ☐ Yes ☒ No

Underwriting Requirements:

1. HbA1c
2. Microalbumin (MICRAL)
3. Blood test for creatinine
4. APS from attending doctor
5. Diabetes Questionnaire



Information from APS:

- Consulted due to fatigue, dizziness and incidental findings of 4+ sugar in urine
- Diagnosed with DM 2 years ago
- Treatment given
- Lost to follow – up

Source : SUN LIFE PHILIPPINES
Clinician : N/A

Date Requested : 03-09-2019 06:51 AM

Received : 03-09-2019 11:02 AM

TEST	Result Reference Interval	(SI Unit)	Result Reference Interval	(Conv. Unit)
HEMATOLOGY				
Glycosylated Hb (HbA1C)	↑ 11.40	%	↑ 11.40	%
			< 5.70	
Comment/s: ADA Recommendation 2015: Pre-Diabetes: 5.70 - 6.40% Diabetes : >= 6.50%				

Information from DBQ

- Diagnosed with diabetes 2 years ago
- Self-medicating, no follow – up done

Source : SUN LIFE PHILIPPINES
Clinician : N/A

Date Requested : 03-09-2019 06:51 AM

Received : 03-09-2019 11:02 AM

Test	Results	Unit	Reference Interval
CLINICAL MICROSCOPY			
MICRAL	100 POSITIVE	mg/L	<20

Thoughts to ponder:

1. Self-medicating
2. Lost to follow – up
3. All test results are above normal

Creatinine (ENZYMATIC)	↑ 130.00	umol/L	↑ 1.47	mg/dL
			0.51~0.95	

What is the underwriting decision?

CASE NUMBER 7



Significant findings:

49 years old
Male
2 years with DM
HbA1c – 11.40%
MICRAL – 100 mg/l
Creatinine – 130 umol/l

Base rating:

+75 DM for age and duration
+100 for MICRAL

However, in view of HbA1c results – more than 10, case will be declined.

Additional consideration:

Creatinine result is elevated

egfr <60 (result: 38.1 mL/min/1.73m²)
Low egfr is also for decline.

Life Rating - Type II (on or off medication)

Note: All TDB is Declined

Age at time of underwriting	Life Duration (in years)		
	<6	6 – 15	> 16
40 – 49	+75	+100	+125

Co-morbidities with sum rating exceeding +200: decline
With only diabetes, if rating exceeds +250: decline
See also [Current Control](#) and [Additional Considerations](#)

Current control

	Life	TDB	CI
Control - HbA1c (%)			
Excellent (HbA1c < 7.1)	May credit -25 or -50 (see below)*		+0
Good (HbA1c 7.1 - 8.0)	+0		+0
Fair (HbA1c 8.1 – 10.0)	+75		Dec
Poor (HbA1c > 10.0)	Dec		Dec

Case is declined.

Albuminuria/proteinuria

Albuminuria ≤ 30mg/L or Proteinuria ≤ 10 mg/L are +0, irrespective of duration
Otherwise, use [Categorization Chart](#) and assess as below (use the higher of the two)

New finding of IGT/IFG	Sum Albuminuria
Otherwise rate as below (DO NOT obtain repeat urine for this purpose alone)*	
A1	< age 55 +0
A2a	+100
A2b	+200
A3	Dec
*If repeat urine is obtained for other purposes, use the Categorization Chart . If the results fall into 2 categories, use the higher of the two.	

Measure		Categorization			
		A1	A2a	A2b	A3
Albumin	mg/dL	< 3	3 - 15	16 - 30	> 30
	mg/L	≤ 30	31-150	151-299	> 300

Hyperlipidemia and Metabolic Syndrome Risk Assessment Guide

Abnormalities of metabolism characterized by elevation of serum cholesterol and/or triglycerides. The metabolic abnormality may be primary (inherited) or secondary to diet, medication or other disorders e.g. diabetes mellitus, hypothyroidism.



- Individuals with hypercholesterolemia are usually asymptomatic. Therefore, **a large proportion of people remain undiagnosed and untreated.**

Client profile:

39 years old

Male

Smoker

Engineer

Earning Ps.1M/Annum

Plan and coverage details:

Applying for:

- Sun Smarter Elite Peso 10 – Ps. 1M
- TDB, ADB - Ps. 1M

Basic Information:

- No significant family history
- 5'9, 180 lbs.
- APE results are within normal limits except for elevated cholesterol and triglycerides
- Taking Crestor tab until 6 months

Underwriting Requirements:

1. Lipid profile – total cholesterol, HDL, triglycerides

EXAMINATION	RESULTS	REFERENCE RANGE/INTERVAL
CHOLESTEROL	8.25	0 - 5.2 mmol/L
LDL	6.29	0 - 3.36 mmol/L
HDL	1.09	1.03 - 1.55 mmol/L
TRIGLYCERIDE	15.15	0 - 1.69 mmol/L
VLDL	6.87	0 - 0.91 mmol/L

What is the underwriting decision?



Significant findings:
Cholesterol – 8.25 mmol/l
Triglycerides – 15.15 mmo/l
HDL – 1.09 mmol/l
Chol/HDL ratio:
8.25/1.09 = 7.56 or 7.60

Rating - Hyperlipidemia

	Life	TDB
Note: For elevated cholesterol, use total cholesterol/HDL ratio for risk assessment, when available. Otherwise, use the total cholesterol.		
Total Cholesterol/HDL Ratio		
Ratio 5.1 to 7.5 All ages	+0	Std
Ratio 7.6 to 9.0		
Up to age 40	+50	Std
Age 41 to 60	+25	Std
Over age 60	+0	Dec

Base rating:

+50 for elevated cholesterol
+100 for elevated triglycerides

We will only choose the higher rating among the two.

There is a debit of 100.

Mortality rating is +100 or 200%.

Underwriting decision is substandard.

Rating - Metabolic Syndrome X or Hypertriglyceridemia or Metabolic Syndrome X with Hypertriglyceridemia

Fasting or Non-Fasting Triglyceride Level		Life		
		HDL Level*		
		(mg/dL (mmol/L))		
		< 33	33 - 40	> 40
mg/dL	(mmol/L)	(< .85)	(.85 - 1.04)	(> 1.04)
≤ 300	(≤ 3.39)	+0	+0	+0
301 - 500	(3.4 - 5.69)	+25	+0	+0
501 - 750	(5.7 - 8.49)	+75	+50	+25
751 - 1500	(8.5 - 17)	Dec	+150	+100
> 1500	(> 17)	Dec	Dec	Dec

RECONSIDERATION PRACTICES



send Review of Decision
request to:

PHIL-ePDS-
Scanning@sunlife.com
and **assigned UW**

- **Request must be submitted within ten (10) days from date of underwriting decision.** This is a complex request and current Turnaround Time to review this type of request is **7 working days.**
If request is considered, requirements for reconsideration of rating should be submitted **within the same deadline** as the Acceptance of Offer requirement deadline.
- **Request for reconsideration of a decline or postpone decision of an application that has been closed in the system.** Take note of the postponement period and the reconsideration requirements to be submitted.
- **Request to reopen NPW (Not Proceeded With) cases. Before requesting to reopen the case, ensure all requirements can be submitted at the same time as extension will not be allowed anymore**

UNDERWRITING DECISION PROFILE FOR 2018

NON-GIO & NON-SIO

Decision	Basic Life		TDB		ADB		CIB (Sun Life Assure & CIB Rider)	
	# of apps approved	%	# of apps approved	%	# of apps approved	%	# of apps approved	%
Standard	184317	94.94%	140936	95.63%	100413	95.21%	3247	92.01%
Rated	1300	0.67%	487	0.33%	394	0.37%	32	0.91%
Dec/Postpone	1407	0.72%	893	0.61%	765	0.73%	68	1.93%
NPW	7115	3.66%	5056	3.43%	3896	3.69%	182	5.16%

06
Our Scorecard

AVERAGE ISSUE AGE FOR 2018

SLOCPI:

Average issue age by policy amount: **31 years old**

Average face amount: **Ps. 663,770**



End of Module



Thank you!

We can start the open-question forum